MODULE 6

The mission depends on the margin.

Financial leadership is not a specialty for the finance team. It's a core responsibility of every practice manager.

Reading financial reports and interpreting them are two completely different skills — and only one of them leads to leadership.

- Revenue flat while AR is rising: something changed in billing or collections, not patient volume.
- Labor cost as % of revenue above 65%: either revenue is under-captured or staffing is misaligned.
- Days cash on hand below 30: one slow month could threaten payroll. This is not a finance team problem.

KEY INSIGHT
What story are your financial statements telling right now that you haven't fully acted on?

Most proposals that get rejected aren't missing a good idea. They're missing a quantified problem.

- 'Our wait times are too long' doesn't get funded. 'Our 22-minute average wait time is costing us 8% of return visits, worth $240K annually' does.
- The cost of inaction is the most persuasive element of any business case — and the most commonly left out.
- ROI framing: for every dollar invested, what does the practice recover, retain, or protect?

KEY INSIGHT
What investment does your practice need to make that hasn't been approved yet? Have you quantified the cost of not making it?

3 REVENUE GROWTH

Most practices can recover significant revenue before they need to grow at all.

- 5-15% of earned revenue is never collected in a typical primary care practice — not because payers refused, but due to operational failures.
- On a $3M practice, that's $150K-$450K in recoverable annual revenue. Operational improvements, not new patients.
- Revenue leakage sources: coding gaps, denial write-offs, no-shows, underpayments, uncollected patient balances.

KEY INSIGHT
Request your denial report broken down by payer and by reason. The top denial cause is almost always addressable.

The most common cost management mistake: cutting visible large costs that are mission-critical while leaving diffuse small costs that add no value.

- Four categories: mission-critical, value-creating, operational support, non-value-adding.
- Protect mission-critical costs. Cutting clinical staff capacity to hit a budget line is a false economy.
- Five questions before cutting: is this a real variance? Volume-driven? Rate-driven? Systemic or one-time?

KEY INSIGHT
What costs in your area are you tolerating that add no value? Most managers can name them when asked directly.

Days cash on hand is the one number that tells you how financially resilient your practice actually is.

- Below 30 days: one billing disruption away from a cash crisis.
- 60-90 days: comfortable operational reserve — the minimum benchmark for most practices.
- Payer concentration above 30% is a structural risk most managers don't actively manage.

KEY INSIGHT
Do you know your practice's days cash on hand right now? If not, that is the first financial leadership question to answer.

FRAMEWORK

The Revenue Leakage Audit

Before acquiring a single new patient, most practices can recover five figures annually by closing these five gaps.

1. Collection rate gap: below 95% means earned revenue never collected.
2. Denial write-offs: denied claims never reworked and written off permanently.
3. No-show revenue loss: empty slots that could have been filled.
4. Underpayment: payer payments below contracted rates, never audited.
5. Patient balance gaps: balances left uncollected after service delivery.

REFLECT ON THIS

Financial leadership is not about reading reports. It's about knowing what to do about them.

If your administrator asked you today: 'What's our biggest financial risk right now?' — what would you say? And how confident would you be in that answer?

TRY THIS

Three financial leadership actions for this week:

1. Find out your practice's days cash on hand. If you don't know it, that's the first gap.
2. Request your denial report by payer and reason. Calculate the annual revenue impact of the top cause.
3. Identify one non-value-adding cost in your area. Estimate its monthly dollar impact.

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